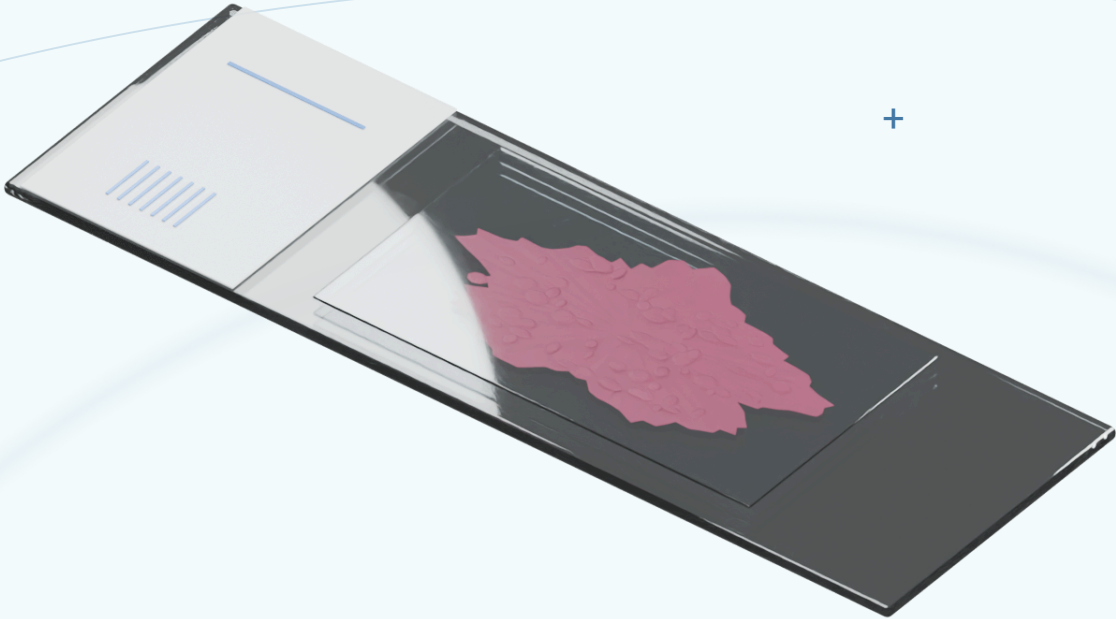


Clarity that scales.

A shared foundation for Artera’s
next chapter.

Mike Jerugim Design Lead



I started with stories. I design for understanding.

WHERE I STARTED

Film & TV post-production.
Recording artist at Interscope.

Sequence. Emphasis. Emotional arc.
Knowing what someone needs to understand next.

This work is personal to me, too.

WHERE I APPLY IT

Kooth / Soluna

Consumer apps and practitioner tools.
Design systems, clinical trust, medical-device application work.

Onix

Consumer and expert experiences.
One foundation across product and brand.

Designing within HIPAA and SOC 2 requirements, with privacy and protection of personal information built into the work.

More contexts. One standard of trust.

THE CHALLENGE

More products & indications

Stop multiplying one-offs.

More clinical complexity

Simplify without losing meaning.

More workflows & markets

Adapt beyond our own environment.

More teams contributing

Make the system the way we work.

THE DESIGN RESPONSE



Build a shared foundation.

Reusable patterns. Explicit variation.



Design for understanding.

Result. Interpretation. Evidence.



Make context configurable.

Roles, handoffs, and local requirements.



Give teams a way to contribute.

Ownership, governance, and adoption.

A proposed approach, grounded in our conversations and the presentation brief.

Audit the friction. Then choose the first win.

A visual inventory tells us what exists.
A workflow audit tells us what matters.

Look across

Reports · portal flows · Figma · code · support themes

Prioritize by

User impact · frequency · rework · business importance

Start with

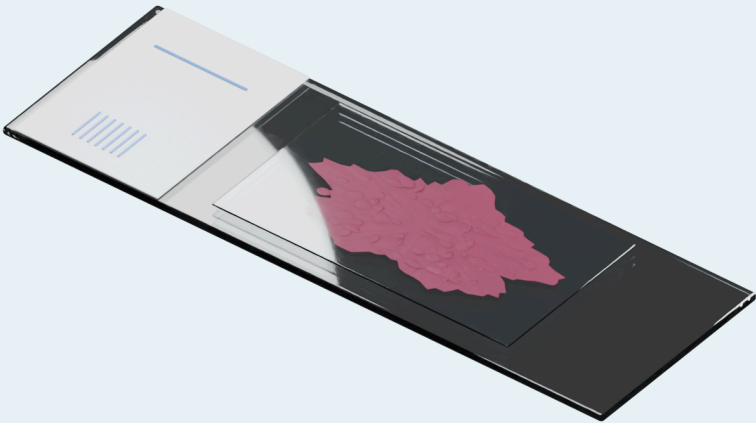
One report pattern. One real workflow. Named partners.

First output A shared map of duplication, risk, and opportunity.

Keep the core. Design for difference.

Shared foundations.
Explicit variation.
Clinical meaning preserved.

The visible experience rests on decisions that should not
be reinvented for every product.

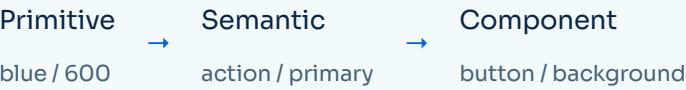
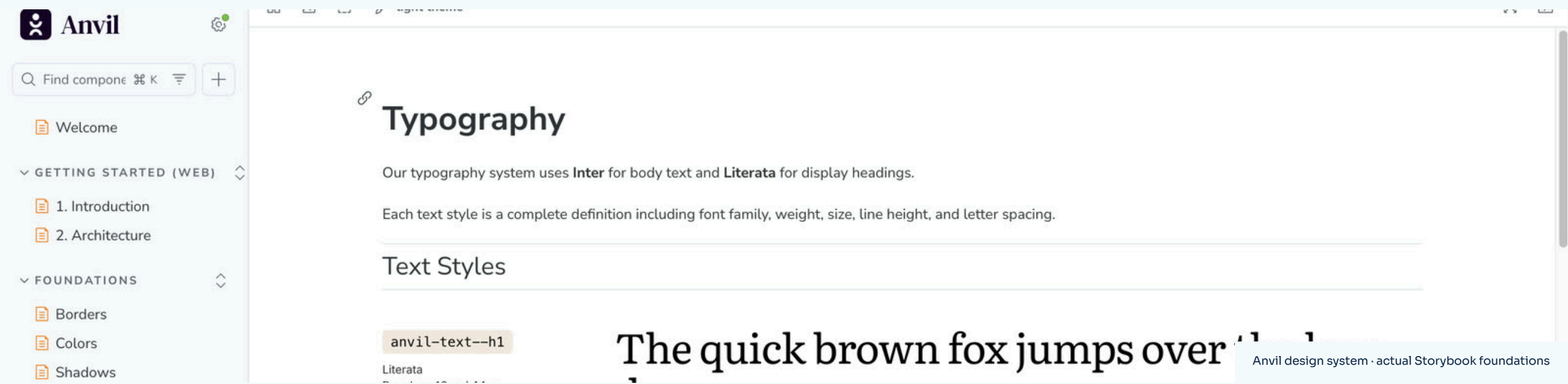


Context	Product · workflow · market
Patterns	Reports · review · recovery
Foundations	Tokens · components · accessibility

A design-system metaphor, not a model of AI inference.

Different surfaces. The same foundation.

At Onix, I led design across the consumer app, expert tools, sites, brand, and design system.

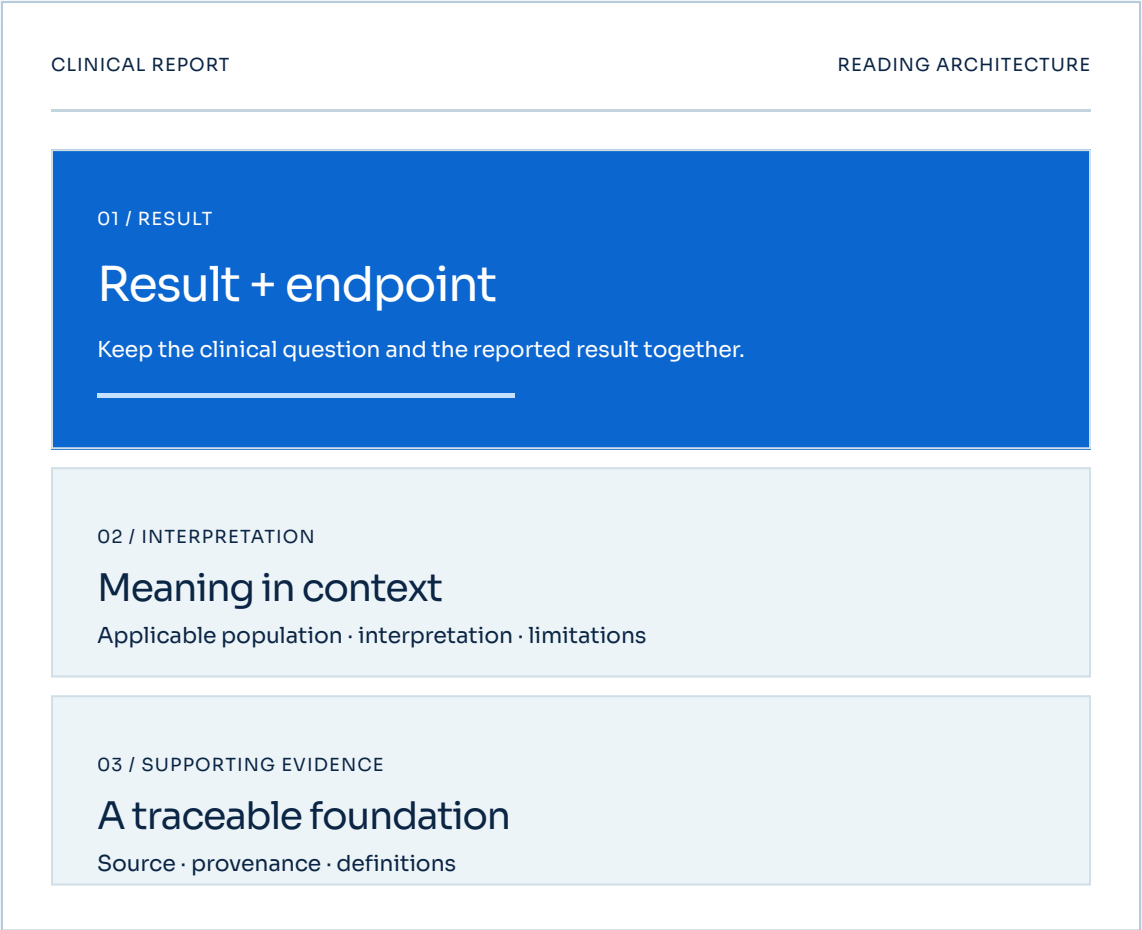


Visual tokens describe intent.
Clinical rules live in explicit schemas.

Same evidence. A clearer story.

The evidence stays intact.
The reading sequence becomes clear.

- 01 **What does it say?**
The result and its clinical context.
- 02 **What does it mean?**
Applicability and limitations.
- 03 **Why should I trust it?**
Evidence and provenance.



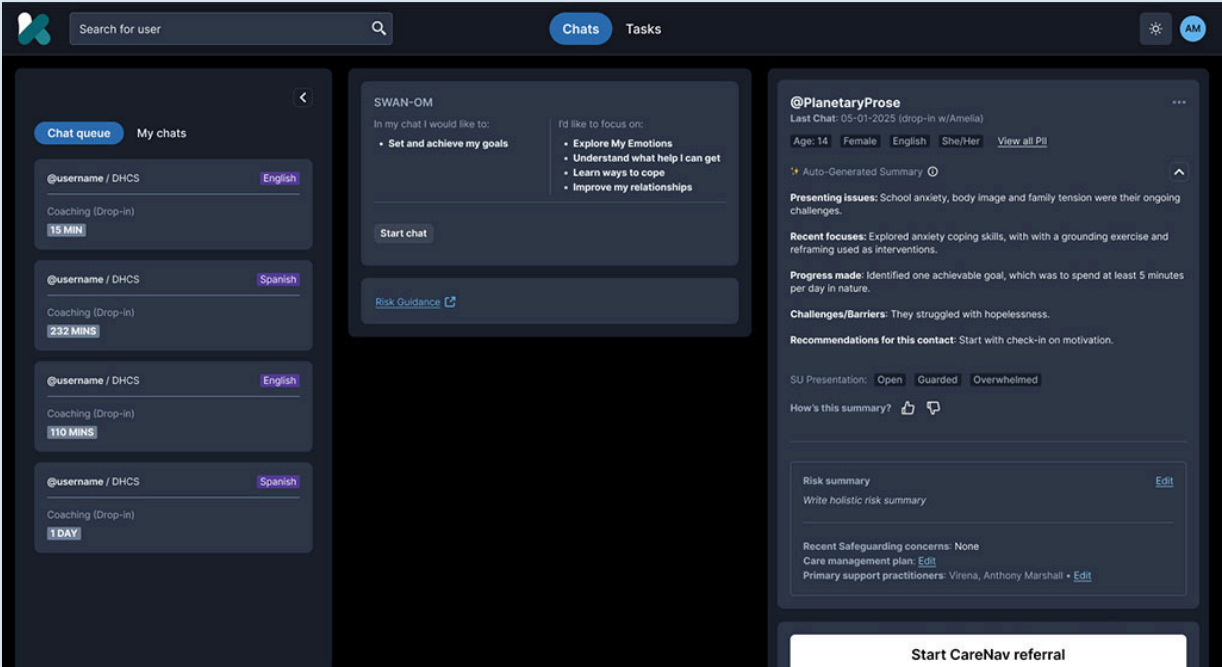
Illustrative structure, not an Artera report redesign or clinical result.

Trust lives in the details.

Helping practitioners enter a conversation with the context they need.

- + Every statement keeps its source.
- + Recency is visible.
- + Data boundaries are explicit.
- + A clinician reviews before use.

Approved for development. Projected savings of 10–15 minutes per session, not a measured outcome.

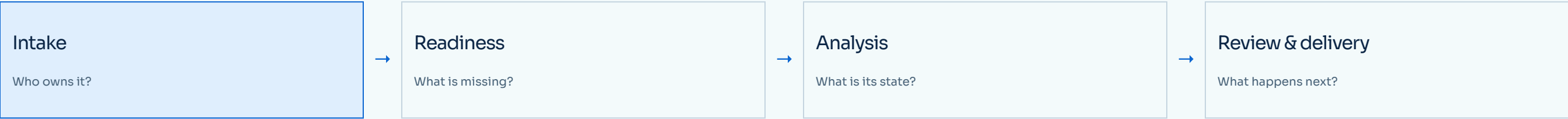


Soluna · practitioner summary design

The transferable principle: make important information easy to find and easy to verify.

Design the handoffs. Not just the happy path.

Pathology teams operate the workflow. Ordering clinicians need the result.
Both need to know what happens next.



[Intake](#) Map the order, responsible role, permissions, and missing information with the pilot lab.

Observe real work with staff. **Standardize** state, ownership, and recovery. **Configure** local integration needs.

Illustrative workflow to validate with Artera and its partners. Cloud-connected analysis, not a claim of local model execution.

Same standard. Different contexts.

Translate the language.
Validate the meaning.

Clinical context

Terminology, applicable evidence, endpoints, and locally supported claims.

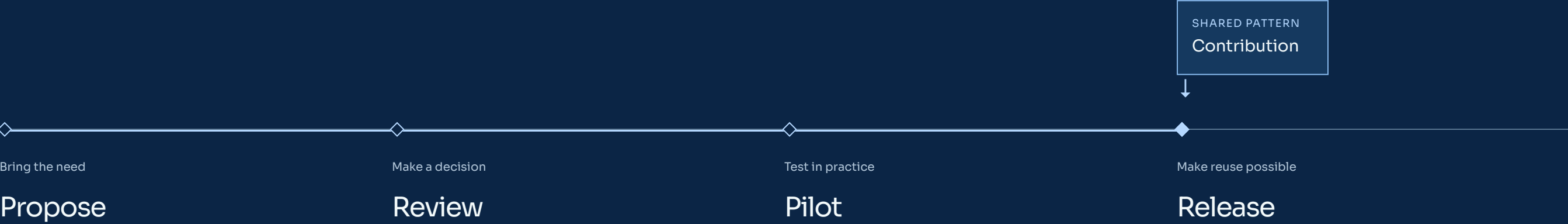
Operational context

Roles, review responsibilities, integrations, and recovery paths.

Human context

Language, reading order, accessible charts, keyboard use, and non-color cues.

A contribution needs a path into the system.



Propose	A problem, evidence, and a named owner.
Review	Design, engineering, and relevant expertise assess the tradeoffs.
Pilot	Test the pattern in real product work. Capture what needs to change.
Release	Document, version, and support adoption. Feed learning back into the system.

When teams disagree

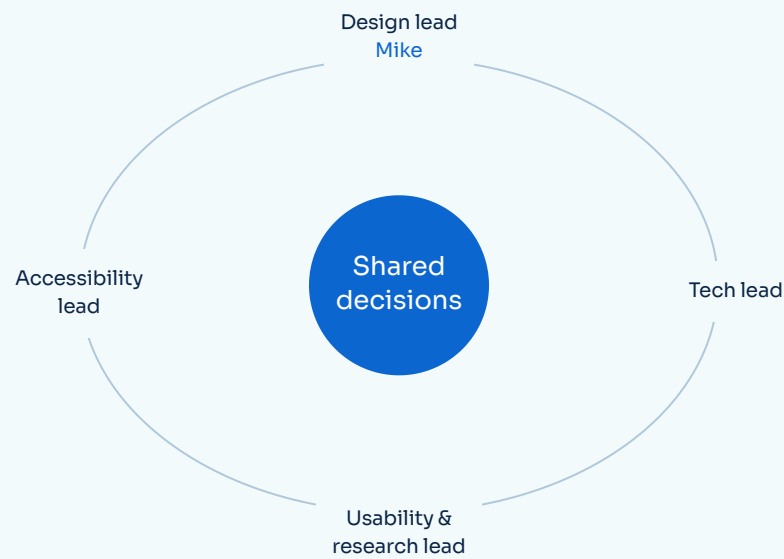
Make user impact, reuse potential, and implementation cost explicit. Name the decision owner. Run a time-boxed pilot when evidence is missing.

Clinical meaning and claims route through the appropriate clinical and regulatory review.

At Kooth, I established the council, contribution model, and documentation framework behind this way of working.

I built the system. And the way we built it.

GOVERNANCE COUNCIL | ESTABLISHED



Also participated in the accessibility working group and served on the AI ethics committee.

Governance

Established the council and ran weekly meetings with designers, researchers, and engineers.

Contribution

Created a contribution model and a documentation framework.

Craft

Built the actual components, tokens, and variables in Figma.

Earn the first team. Then make reuse easier.



Co-build a pilot

Pair with the team on a problem they already need to solve.



Remove the friction

Design and code examples, office hours, migration support.



Show the value

Share what shipped, what improved, and what needs changing.

Adoption is an outcome.
It is not an announcement.

Fund migration in product work.
Agree deprecation windows.
Keep contribution accessible.

Understand. Prove. Scale.

Days 1–30	Understand the work Decision: where to start	Audit, observe, baseline. Choose the pilot and its partners.
Days 31–60	Prove a thin slice Decision: what holds up	Co-design, test, implement. Document the first reusable patterns.
Days 61–90	Scale what works Decision: what to invest in next	Measure, refine, support adoption. Prioritize the next context.

Scope depends on engineering capacity, clinical access, partner availability, and product priorities.

Prove the value in the work.

Delivery

Comparable change lead time
Review iterations and rework

Less repeated effort

Adoption

Eligible work using shared patterns
Active contributing teams

Reuse that actually happens

Experience

Comprehension and task success
Accessibility defects and support themes

Clearer, more usable products

Leadership update

What shipped → who uses it → what improved → what’s next

Translate measured effort saved into an explicit cost estimate.
Keep assumptions visible. Include maintenance.

A shared foundation.
Clear clinical meaning.
A system teams use.

I would start with your teams,
choose a meaningful pilot,
and prove the value together.